

Our Ref: (T) MHC/KS/ [MED-ID]

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

Consultant Respiratory Physician

G Level

West Wing

Mailpoint 63

Division D

Care Group - Cardiovascular and Thoracic Thoracic Surgery

Southampton, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear Anastasios

MR [NAME] , [ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Many thanks for asking me to see this 57 year old gentleman who attended the Outpatient Clinic with his wife and his daughter.

He presented to his GP with a history of cough since November 2014 and was referred for a chest X-ray then up to your clinic. He has been getting more short of breath on exertion and feels tired. CT scan has demonstrated mediastinal and bilateral hilar lymphadenopathy and multiple nodules throughout the lungs with some consolidation in the right middle lobe. A PET scan has shown all these areas to be metabolically active. He has gone on to have an EBUS of his mediastinal node which were nondiagnostic.

Mr [NAME] get more breathless on exertion. He can still work as a lorry driver. He has never smoked and may have been exposed to asbestos.

He is known to have had Crohn's disease but currently takes no regular medication. He has no significant weight loss and review of symptoms is unremarkable. He does suffer with some heartburn.

On examination he looks well in himself. [NAME] is no lymph nodes in his neck. His BMI is 30. We discussed the imaging and the fact that this looked highly suspicious for sarcoidosis but without a tissue diagnosis. We have tried to ascertain this by EBUS which is the least invasive procedure but this has failed. We have therefore discussed a bronchoscopy and a cervical mediastinoscopy as the next steps to obtaining a tissue diagnosis. I have quoted him a less than 1% risk to life along with pain, bleeding requiring a sternotomy and hoarse voice. He understands that if we can ascertain a tissue diagnosis then he can be treated in the most appropriate way.

He understands the risks and is keen to proceed and we will therefore pre-assess him today and place him on my waiting list.

We did discuss the radiology last night in the lung MDT when it was felt that this was highly suspicious for sarcoidosis and a mediastinal lymph node biopsy would suffice. We do not at this stage need to consider biopsying the lung nodules.

Many thanks for your referral.

Yours sincerely

Mr [NAME]

cc:

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Mr [NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]